

--SUMMARY--

Decision No. 714/20

16-Sep-2020

A.Somerville

- Permanent impairment {NEL} (redetermination) (significant deterioration)
- Significant contribution (of compensable accident to subsequent disability)

No Summary Available

15 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4620n]

Style of Cause:

Neutral Citation: 2020 ONWSIAT 1574



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 714/20

BEFORE:

A. Somerville: Vice-Chair

HEARING:

June 11, 2020 at Toronto
Oral by Teleconference

DATE OF DECISION:

September 16, 2020

NEUTRAL CITATION:

2020 ONWSIAT 1574

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) Chow decision dated April 25, 2017

APPEARANCES:

For the worker:

T. Shogren, Union Representative

For the employer:

Not participating

Interpreter:

N/A

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

505, avenue University, 7^e étage
Toronto ON M5G 2P2

REASONS

(i) Introduction

- [1] The worker appeals a decision of ARO Chow, which upheld the denial of right hip entitlement, and the rescission of entitlement for the right hip surgery of November 27, 2009 and associated Loss of Earnings (LOE) benefits. Partial LOE benefits after the subsequent right hip replacement surgery were denied, as were partial LOE benefits beyond the final LOE review date. A recurrence on January 7, 2012 was denied, as was a redetermination of the Non-economic Loss (NEL) award for the worker's low back condition.

(ii) Issues

- [2] The issues under appeal are as follows:
1. Entitlement to right hip injury and the right hip surgery of November 27, 2009;
 2. Entitlement to Loss of Earnings (LOE) benefits following the right hip replacement surgery;
 3. Entitlement to Loss of Earnings (LOE) benefits beyond the 72-month mark; and from January to October 2010;
 4. Entitlement to a redetermination of the 19% Non-Economic Loss (NEL) award for the low back; and
 5. Entitlement to a January 7, 2012 recurrence of the low back injury.

(iii) Background

- [3] The now 64 year old worker, a registered practical nurse (RPN) at a nursing home, suffered low back pain on June 14, 2005 when she was repositioning a resident with a co-worker. The Board accepted entitlement for a low back strain and in November 2006 granted a Non-Economic Loss (NEL) award for a permanent impairment in her low back.
- [4] On February 15, 2007 and January 18, 2008 the worker slipped on the floor at work without falling. Both of these incidents were accepted as recurrences of the 2005 low back injury and were amalgamated into the 2005 back claim.
- [5] It is the worker's position that the slips injured her right hip. In November 2009 the worker underwent right hip arthroscopic surgery and the Board granted full LOE benefits from January 2010 to October 2010 under the low back claim. She underwent right hip replacement surgery in October 2010, for which the Board denied entitlement.
- [6] A Board Medical Consultant (MC) subsequently opined that the worker's right hip condition was not related to the January 2008 slip at work. The Board then rescinded entitlement for the right hip, and for LOE benefits from January to October 2010. The overpayment was not recovered. The Board further concluded that the right hip replacement surgery was also not related to the January 2008 slip at work. The Board determined that the modified duties provided

by the employer were suitable for the worker's low back condition, and the worker was not entitled to LOE benefits after the right hip replacement surgery, or after the final LOE review.

[7] The Board also found that there was not a recurrence of the low back condition on January 7, 2012 as there was not a significant deterioration below the 19% NEL award for the low back.

[8] The ARO's decision indicated that it was the worker's position that the 2007 and 2008 slips were a significant contributing factor to the worker's right hip injury, and that there should be entitlement for the 2009 right hip surgery, and for the 2010 total hip replacement, with associated LOE benefits and a NEL award. The worker also argued before the ARO that she experienced severe low back pain in 2012, that her condition was below her NEL award, and that she was entitled to LOE benefits from January 9 to 17, 2012.

[9] ARO Chow denied entitlement for the right hip condition and associated benefits, due to the minor nature of the slips without falls, the delay in reporting hip symptoms, and the degenerative changes and damage in the hip. The ARO found no entitlement to partial LOE benefits following the hip replacement surgery, or beyond the final LOE review, because the accident employer continued to offer full-time suitable modified duties for the low back impairment, which the worker performed prior to the hip surgeries.

[10] With respect to the worker's position that she suffered a recurrence in 2012 and was entitled to a NEL redetermination, the ARO found that the worker indicated that she suffered severe low back pain in 2012. The ARO found that she also suffered severe pain at the time of her NEL assessment, and therefore she did not suffer a significant deterioration in her low back condition in 2012 beyond the 19% NEL award. The ARO therefore determined that she did not suffer a recurrence and that she was not entitled to a redetermination of her NEL award.

[11] The worker now appeals the ARO decision to the Tribunal.

(iv) Law and policy

[12] Since the worker was injured in 2005, 2007 and 2008, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[13] Pursuant to section 126 of the WSIA, the Board stated which policy packages, Revision #9, would apply to the subject matter of this appeal. I have considered these policies as necessary in deciding the issues in this appeal.

(v) Analysis

[14] The appeal is allowed in part for the reasons below.

(a) Entitlement to right hip injury, the right hip surgery of November 27, 2009, and the 2010 right hip replacement surgery

[15] I find that the worker has entitlement for the right hip injury, the right hip surgery of November 27, 2009 and the 2010 right hip replacement surgery for the reasons below.

[16] With respect to questions of causation, Tribunal jurisprudence applies the test of significant contribution. A significant contributing factor is one of considerable effect or

importance. It need not be the sole or main contributing factor. See, for example, *Decision No. 280*.

- [17] The worker's representative submitted that the worker's right hip injury was due to her compensable accidents and that the worker therefore should be granted entitlement for her right hip impairment and related surgeries. She submitted that the worker should be granted entitlement to a NEL determination for her right hip impairment, plus LOE and health care benefits.
- [18] These were also the worker's representative's submissions before the ARO. The ARO found that the worker's hip condition was not compatible with the 2008 accident and therefore denied entitlement for the right hip impairment, for the 2009 surgery and any associated benefits. He also found that the modified work offered by the accident employer was suitable for the worker's compensable back condition and therefore denied entitlement for LOE benefits following the 2010 hip replacement and at the final LOE review. Given the ARO's finding that the right hip injury was not compensable, I find that he implicitly decided that there was no entitlement for the 2010 right hip replacement surgery and any related benefits. I find that I therefore have jurisdiction over the issues of entitlement for the 2010 hip replacement surgery and entitlement to a NEL award.
- [19] The worker testified she was a staff nurse at the accident employer, supervising eight to ten personal support workers (PSWs). She supervised residents' treatment, medications and dressings. She usually worked evenings, and the PSWs did most of the hands-on care, but she assisted when necessary. She testified that her back was injured when she was assisting with lifting a resident to reposition him in order to change a dressing. When she returned to work, she did her regular job but was responsible for fewer residents. Her back pain was constant, but she did not have hip pain.
- [20] She testified that on February 15, 2007 when exiting a resident's room, she slipped on some water left by a housekeeper who was washing floors. She did not fall, but her right leg went out and she jarred her back and her groin.
- [21] She testified that she reported the incident to the nurse in charge and sought medication from her family doctor, Dr. Tiejde, the next day. I note that Dr. Tiejde provided a Health Professional's Report (Form 8) dated February 16, 2007 that diagnosed a twisting injury to her low back. In a Worker's Continuity Report from February 2007 the worker wrote that her right low back was jarred, and she felt pain across all her low back, both buttocks, and lateral right thigh to her knee. Her Worker's Report of Injury/Disease (Form 6) dated March 2007 indicated that she injured her low back and right buttock. The worker indicated that she was off work for about a week, and when she returned, her job was changed to doing audits. She performed this work with difficulty as her back and hip were sore. She testified that her groin started to "seize up" and sometimes she could not put weight on her right leg, but she did not realize the problem was in fact her hip.
- [22] A report from a hospital emergency department in December 2007 stated that since December 14, 2007 her pain in her low back radiating into her right buttock to her right anterior thigh and right groin had increased. There was no specific injury on December 14, 2007 and the worker reported a gradual onset of the right anterior thigh pain since that date. She was unable to full weight bear on her right leg and had concerns about falling.

- [23] The worker testified that in January 2008, while doing her regular duties, she again slipped on a puddle of water on the floor. Her right leg went out again, she jarred her right hip and back, but she did not fall as she grabbed onto a door.
- [24] The worker's Form 6 from the same month indicated that she injured her low back and right groin. A letter from the worker dated January 30, 2008 was attached to the Form 6, stating that she had left and right low back pain into her buttocks and posterior thighs since her June 2005 and February 2007 work injuries. The letter further indicated that on December 14, 2007 she had pain across her low back and she suddenly developed pain in her right anterior upper thigh and right groin. When she returned to work for three days starting on December 15, she could not put her total weight on her right leg and had several episodes of pain in her right groin.
- [25] I drew this letter to the worker's attention at the hearing, and she testified that she did not know when her groin pain started, but it was after the 2007 slip, and she had it throughout 2007 several times per week. She further testified that she thought the groin pain was part of her back injury, and she did not know that groin pain was a hip problem until a physiotherapist explained this to her in 2008. I accept the worker's sworn testimony that she had groin pain periodically since the 2007 accident, and I accept her explanation that there was confusion about this, as she did not understand that groin pain was a symptom of a hip injury.
- [26] The worker testified that she was off work for a few days after the 2008 slip, and then she tried to return to work but she had difficulty walking due to pain. She was then off work from about March 2008 until January 2011 when she returned to work with the accident employer.
- [27] An arthrogram of the right hip on September 2, 2008 revealed degenerative changes within the superior labrum and no definite tear, and acetabular cartilage chondropathy.
- [28] The worker had a right hip arthroscopy and labral chondral debridement on November 27, 2009. The pre-operative diagnosis was "labral chondral damage, right hip" and the post-operative diagnosis was the same, as well as severe chondral damage with a large chondral flap. The worker testified that the arthroscopy did not relieve her symptoms, and a total right hip replacement was performed in the fall of 2010. She also testified that she had a further right hip replacement at a later date, and I note that the issue of entitlement for this further surgery was not before me.
- [29] In reaching my conclusion that the worker has entitlement for a right hip injury and for the 2009 arthroscopy and 2010 hip replacement I give weight to the opinion of Dr. Beaulé, the worker's orthopaedic surgeon. He reported on September 23, 2010 that the worker was scheduled for a total right hip replacement due to advanced arthritis. He further opined:
- She did sustain injury in slippage in 2007 at work and then reinjured it in 2008 at work again and subsequently developed arthritis due to cartilage tearing. It is not due to her lower back injury sustained in June 2005.
- [30] In July 2019 Dr. Beaulé responded to a number of questions from the worker's representative. He indicated that a whole body scan of the worker taken in December 2005 did not show degenerative changes in her right hip. He opined that a slip without a fall could cause a hip injury, because an abrupt motion or force could lead to a cartilage tear. Finally, in response to the question "in your medical opinion what was the significant contributing factor to [the worker's] hip condition," Dr. Beaulé responded "cartilage tear from slip at work."

[31] I give weight to Dr. Beaulé's opinion because he was the orthopaedic specialist who treated the worker and performed her surgery. His letterhead indicates that he is the Head of Adult Reconstruction Services, with a specialty in "joint preserving surgery of hip" and "joint replacement surgery hip and knee" at a city hospital. As such, I find he was imminently qualified to opine on the nature and cause of the worker's hip impairment, and his opinion is deserving of weight.

[32] Further, my conclusions that the worker has entitlement for her hip impairment and related hip surgeries are fortified by the following:

- A December 2005 whole body scan contains no findings with respect to the right hip. As indicated above, Dr. Beaulé confirmed that the scan showed no degenerative findings in the right hip. I find it significant that there were no right hip findings in this scan conducted prior to the 2007 and 2008 slips at work.
- In a Health Professional's Progress Report (Form 26) dated March 2008, Dr. Tiedje noted an increase in right hip pain, now worse than the back, and questioned whether there was a hip injury at the same time as the back injury.
- Physiotherapist T. MacKenzie reported in March 2008 that the worker had a flare up of her right hip pain which was an associated problem with her back.
- In his report of April 24, 2008 Physiotherapist MacKenzie opined that her right hip findings were consistent with early osteoarthritis and that her slips on wet floors may have been causative factors.
- Dr. Allen examined the worker on June 13, 2008 and reported that she had intermittent right groin pain that "seems to have stemmed from a couple of falls."
- The Board's Medical Consultant Dr. Steinnagel opined in November 2010 that the worsening of the hip was likely in part a sequelae of the 2008 slip and therefore the subsequent surgery and hip replacement appeared to be related to the secondary entitlement granted albeit on an aggravation basis, but did not appear to be directly related to the low back injury.
- In a report dated August 2011 to the worker's representative, Dr. Tiedje indicated that the worker never had complaints of right hip pain prior to the 2007 and 2008 work accidents, nor did she walk with a cane or limp. My review of the medical reports also confirms that the worker's pain was primarily left sided before the 2007 slip. Dr. Tiedje further stated that "in my limited experience, I would have to surmise that her ultimate hip surgery was at least partly a consequence of the work injury." He also reported that the worker required a total hip replacement "due to her severe osteoporosis". When I questioned the worker about this, she testified that she did not have osteoporosis and that she believed that Dr. Tiedje stated this in error. As there is no other reference to osteoporosis in the case materials, I accept this explanation.

[33] There is also a report from Board medical consultant, Dr. Gelman dated June 2016, stating that it was difficult to say for certain whether the hip issues were due to the 2008 injury or related to other factors such as degenerative changes. Dr. Gelman opined, however, that it was more likely that the hip issues were not related to a work injury given that the minor nature of the injury, consisting of a slip without a fall, would be unlikely to cause labral damage. As well, he

noted that there was a delay in reporting hip symptoms from this injury, which would not seem consistent with an acute labral tear. He noted that the September 2, 2008 MR arthrogram did not show evidence of a definite tear. He also opined that the presence of degenerative changes and the apparent relationship of the chondral damage due to degeneration would mitigate against this being related to a minor acute injury. He therefore concluded that the right hip/labral area injury was not compatible with the history of the 2008 incident, and the 2009 surgery was also not compatible.

[34] In evaluating the weight to be given to Dr. Gelman's report, I observe that his report indicates that he is a specialist, but does not specify the nature of his specialty. I also observe that he did not examine or treat the worker. For these reasons, I give more weight to Dr. Beaulé's report as to the cause and nature of the worker's right hip impairment, including his opinion that the worker had a cartilage tear due to the 2007 and 2008 slips at work, as he was the worker's treating orthopedic specialist, with a specialty in hip surgery, who performed both her surgery in 2009 and her 2010 hip replacement.

[35] In summary, I find the evidence establishes on the balance of probabilities that the worker's 2007 and 2008 slips at work were significant contributing factors to the worker's right hip injury. I find that the evidence establishes on the balance of probabilities that her compensable right hip injury was a significant contributing factor to her 2009 arthroscopic surgery and her 2010 hip replacement. I find therefor that her hip replacement surgery was compensable, and I am satisfied that the worker's compensable right hip injury resulted in a physical or functional abnormality or loss for which she is entitled to a NEL assessment. In reaching this conclusion, I observe that there are potentially two components to a rating for the hip in the AMA Guides: range of motion and other disorders of the hip, including those noted in Table 45. I particular, I observe that Table 45 of the AMA Guides entitled "Impairment of the Lower Extremity Due to Other Disorders of the Hip Joint", sets out a rating for a replacement arthroplasty.

[36] I therefore find that the worker is entitled to a NEL assessment for her compensable right hip impairment.

(b) Entitlement to Loss of Earnings (LOE) benefits following the right hip replacement surgery, and

(c) Entitlement to Loss of Earnings (LOE) benefits beyond the 72-month mark; and from January to October 2010;

[37] I find that the worker is entitled to full LOE benefits from January to October 2010 and full LOE benefits from the date of her right hip replacement surgery in October 2010 until her return to work in January 2011. I refer the issues of her entitlement to LOE after her return to work in January 2011 and at the final LOE review back to the Board to determine. My reasons are as follows.

[38] Section 43 of the WSIA provides in part that:

43(1) A worker who has a loss of earnings as a result of the injury is entitled to payments under this section beginning when the loss of earnings begins. The payments continue until the earliest of,

(a) the day on which the worker's loss of earnings ceases;

- (b) the day on which the worker reaches 65 years of age, if the worker was less than 63 years of age on the date of the injury;
- (c) two years after the date of the injury, if the worker was 63 years of age or older on the date of the injury;
- (d) the day on which the worker is no longer impaired as a result of the injury. 1997, c. 16, Sched. A, s. 43 (1).

...

(3) The amount of the payment is 85 per cent of the difference between his or her net average earnings before the injury and any net average earnings the worker earns after the injury, if the worker is co-operating in health care measures and,

- (a) his or her early and safe return to work; or
 - (b) all aspects of a labour market re-entry assessment or plan. 1997, c. 16, Sched. A, s. 43 (3); 2000, c. 26, Sched. I, s. 1 (6).
- (4) The Board shall determine the worker's earnings after the injury to be the earnings that the worker is able to earn from the employment or business that is suitable for the worker under section 42 and is available and,
- (a) if the worker is provided with a labour market re-entry plan, the earnings shall be determined as of the date the worker completes the plan; or
 - (b) if the Board determines that the worker does not require a labour market re-entry plan, the earnings shall be determined as of the date the Board makes the decision. 2007, c. 7, Sched. 41, s. 2 (2).

[39] OPM Document No. 18-03-02 "Payment and Reviewing LOE Benefits (Prior to Final LOE Review)," explains the circumstances in which "Treatment with No Return to Work" is appropriate:

If the nature or seriousness of the injury completely prevents a worker from returning to any type of work, the worker is entitled to full LOE benefits, providing the worker co-operates in health care measures as recommended by the attending health care practitioner and approved by the WSIB. If the worker does not co-operate, the WSIB may reduce or suspend the worker's LOE benefits.

[40] As indicated above, the worker testified that she was off work from March 2008 until January 2011 due to back, hip and groin pain which affected her ability to walk. She seeks entitlement to LOE benefits for the period from January 2010 to October 2010. She testified that the Board paid benefits during this time, and later rescinded entitlement, but there was no overpayment. I note that ARO Chow's decision stated that entitlement to full LOE benefits was originally granted following the November 2009 right hip arthroscopic surgery, but when entitlement for that surgery was rescinded, entitlement to the full LOE benefits was also rescinded. The overpayment was not recovered.

[41] Since I have found that there is entitlement for the November 2009 surgery, I am restoring entitlement to the benefits from January 2010 to October 2010 as originally granted by the Board. However, because the Board did not recover the overpayment, and the worker has already received these benefits, there is not entitlement to further benefits during this time period.

[42] The worker testified that after her October 2010 right hip replacement she returned to modified work in January 2011 with a cane, and for shorter hours. She testified that she had more restrictions with the added hip impairment than those that she had for her back injury

alone. I note that the Board Memo# 34 dated April 2006 from Dr. DesLauriers opined that her restrictions due to her compensable back injury included no forceful repetitive bending or twisting of the lumbar spine, and that she should be very careful with lifting, walking, standing, sitting, and stair climbing as tolerated. She was to avoid a prolonged static position of the lumbar spine, and should alternate sitting, standing and walking. The worker testified that when she returned to work in January 2011 it was her understanding that she was to avoid extensive walking, sitting, standing, bending, twisting, pushing and pulling. I was not directed to a document describing her restrictions after the hip replacement surgery.

- [43] The worker indicated that when she returned to work she was a restorative care coordinator, which involved monitoring PSWs and helping train residents to become more independent. She was expected to assist with the work, if needed. After a while, the work changed, and she did the “RAIMDS” job, which involved collecting data about the residents which was relevant to a new way of funding, and entering the data on the computer. She spoke with the residents when she did this job and learnt computer skills.
- [44] She testified that later, the accident employer asked her to choose between these two jobs, and she chose to do the RAIMDS job. However, she indicated that she was told to do the restorative care coordinator job, which she accepted as she was afraid of losing her job. She testified that she had no formal training for this job. It required her to work with the residents, and involved lifting, stooping, and bending. She testified that she could not do the job physically or emotionally and it did not go well. She had constant pain in her back and left side.
- [45] She testified that she thought that she was working part time hours but at some point returned to full time hours, although she could not remember when that was. She indicated that the accident employer told her that it could not keep her if she was working part time, and asked her to work full-time.
- [46] The worker testified that she stopped working in September 2013 after meeting with a psychiatrist when she was distraught and in pain. She was diagnosed with a major depression, anxiety and pain. She officially retired in June 2014 and thought that she was on short-term disability from September 2013 to April 2014.
- [47] The worker's representative submitted that the restorative care coordinator job was not suitable for the worker's restrictions and she was bullied into this position. The representative did not make submissions concerning the quantum of LOE benefits at the final LOE review date, although she was invited to do so.
- [48] I am satisfied that the worker did not have the capacity to work following her compensable October 2010 hip replacement due to her compensable injury. The worker therefore is entitled to full LOE benefits following her hip replacement surgery until her return to work in January 2011.
- [49] As indicated above, I have found that the worker's right hip condition and her surgeries are compensable, and I have directed the Board to conduct a NEL assessment for her right hip permanent impairment. Given that the quantum of the NEL award is not known until that assessment occurs, that the Board has not had an opportunity to consider the suitability of the work offered by the employer in January 2011 and thereafter in light of my findings that the right hip impairment is compensable, and given I was not referred to a document containing the worker's restrictions following the hip replacement surgery, I am referring the issues of

entitlement to LOE benefits following the worker's return to work in January 2011 and at the final LOE review date back to the Board for the Board to determine, subject to the usual rights of appeal.

(d) Entitlement to a redetermination of the 19% NEL award for the low back

[50] I find that the worker is not entitled to a redetermination of her 19% NEL award for her low back for the reasons below.

[51] Section 46 of the *Workplace Safety and Insurance Act, 1997* (the WSIA), as amended, provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss. "Impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss. "Permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery.

[52] Legislation and Board policy provide that the degree of a worker's permanent impairment is determined in accordance with the prescribed rating schedule or criteria, any medical assessments, and having regard to the health information on file. The prescribed rating schedule for most impairments is the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised) (the AMA Guides).

[53] Subsections 47(9) and (10) of the WSIA provide that if a worker with a permanent impairment greater than zero suffers a significant deterioration in his or her condition, and 12 months have passed since the Board's most recent determination concerning the degree of impairment, the worker may request that the Board redetermine the degree of the worker's permanent impairment. Board *Operational Policy Manual* (OPM) Document No. 18-05-09, "Redeterminations" defines the term "significant deterioration" as follows:

A significant deterioration refers to a marked degree of deterioration in the work-related impairment that is demonstrated by a measureable change in objective clinical findings.

[54] The worker's representative submitted that the worker's compensable low back condition had deteriorated primarily as evidenced by her worsening pain in January 2012, and that her NEL award should be redetermined on that basis.

[55] The worker's NEL award was rated on November 28, 2006. The diagnosis noted was chronic mechanical lower back pain. The medical assessor noted that she had constant steady aching pain with the occasional severe electric current pain in the left buttock, and occasionally radiation down the leg. She could only sit for 3 to 15 minutes, and aggravating factors included twisting, lifting, and walking. She was tearful and depressed with the situation. The worker's range of motion in her dorso-lumbar area was restricted, with flexion of 40°, extension of 5°, right lateral flexion of 10° and left lateral flexion of 20°. The abnormal motion was rated at 15%. There was also a 5% rating for an unoperated intervertebral disc or other soft tissue lesion with a minimum of six months of medically documented pain, recurrent muscle spasm or rigidity associated with none to minimal degenerative changes. The 15% and 5% ratings were combined to produce a 19% rating.

[56] The case materials indicated that the worker has experienced pain since her June 2005 back injury. For example, in September 2005 Dr. Giaccone, a neurologist, noted that since the work accident the worker described a burning, stinging pain that persisted. A progress report

from the worker in the same month stated that she was still having left low back pain, spasms, and she had recently developed sciatica into her left buttock. Dr. MacGregor, physical medicine and rehabilitation, reported in December 2005 that the worker had left upper buttock pain that was a dull ache most of the time, but it could be throbbing and varied in intensity from 2/10 to 7/10 on an average day.

[57] In February 2011, Dr. Kingwell, orthopaedic surgery, examined the worker “in follow up regarding her severe mechanical low back pain.” He reported that she had severe low back pain with no radiation into her legs. He opined that she was unable to work more than she was at that time, which was four hours per day on a varying schedule of days on and off as described in his report. She had limited forward flexion, and could only extend to neutral due to severe back pain. Neurologic examination was normal in her lower extremities. Straight leg raises were approximately 45° bilaterally. Her reviewed her 2008 MRI, and opined that it demonstrated disc degeneration, some facet arthropathy with no neurologic compression. His diagnosis was severe mechanical back pain.

[58] In his August 2011 report to the worker's representative, Dr. Tiedje indicated that the worker had chronic back pain, weakness and endurance, all of which limited her to five hour shifts at work.

[59] In an October 2011 report, Ms. Simmons, a registered massage therapist, indicated that the worker sought treatment in July 2011 due to numbness and pain in her right leg that had begun in late June and got worse in July to the point that she could not walk without a cane. In September 2011 she sought treatment again due to severe upper and low back pain. In late September 2011 her right leg pain was bad, she needed a cane, and her low back was painful.

[60] The worker testified that on January 7, 2012 her back flared up without a specific event occurring at work. She was working with the residents, and she suffered severe back pain and her back seized up. A PSW got a wheelchair and she went to emergency. She could not remember whether she lost time from work, but it might have been a day or two. She testified that her back pain was always worse after then, with spasms. She treated the pain with pain medication.

[61] Dr. Cybolski treated the worker at a hospital emergency department on January 8, 2012. His report is difficult to read but it appears that he diagnosed chronic mechanical low back pain with sciatica. Straight leg raising was 90° on the right and 70° on the left side. He indicated that she should work when she was able to, and should follow up with her family doctor. I note that the doctor's findings with respect to straight leg raising were better than those noted by Dr. Kingwell in February 2011.

[62] The worker's Form 6 dated January 18, 2012 stated that she was doing restorative care duties with the residents on January 7, 2012 and her back was more sore than usual. The pain exacerbated over the next hours with severe pain in the left low back, left buttock and left leg. She had shooting pain and weakness in the left leg and was not able to put full weight on that leg. I was not directed to any medical reports authorizing her to be off work.

[63] The representative also indicated that the worker suffered a lumbar strain in October 2012 when she stopped a resident from falling. I observe that the case materials contain emergency department records from October 2012 indicating the worker developed sudden low back pain radiating to her right leg after stopping a resident from falling. The worker had

difficulty ambulating but was able to move both legs well. The attending doctor reported that there was a history of lumbar facet joint pain and the worker had been doing well since an August cortisone injection. The Board allowed entitlement for a new accident with a diagnosis of acute strain, and LOE benefits were paid.

[64] The worker's representative further indicated that the worker injured her low back again in August 2013 when she was kicked by a resident. The case materials indicate that entitlement was allowed for a low back contusion and exacerbation of chronic mechanical low back pain, and LOE benefits were paid by the Board.

[65] As indicated above, the worker's representative submitted that the worker is entitled to a NEL redetermination due to her worsening pain since her 2006 NEL assessment, primarily in January, 2012. I note that the worker's NEL award is for an organic impairment. With respect to organic impairments such as the worker's, it is important to understand that the rating obtained using the AMA Guides, including those for reduced range of motion, reflects the level of pain normally associated with the objective findings related to the organic impairment. In other words, for organic impairments, there is not a separate rating for pain. *Decision No. 1850/09* stated that as follows in this regard.

...when the AMA Guides are used to provide a rating of an organic impairment, the rating is intended to reflect a level of pain that is normally associated with the objective findings related to the particular level of organic impairment. If the extent of pain that a worker experiences exceeds that which would normally be associated with the organic findings, the pain is said to be "inconsistent with the organic findings" and the worker may be entitled to an award on a non-organic basis (i.e., an award for chronic pain disability (CPD)). In this appeal, the Board has not ruled on whether the worker has entitlement on a non-organic basis, and such entitlement is not an issue before me in this appeal.

[66] Board policy defines "significant deterioration" as "a marked degree of deterioration in the work-related impairment that is demonstrated by a measureable change in objective clinical findings". Again, the worker's position was primarily that she was entitled to a redetermination of her NEL award for organic impairment based on her experience of increased pain. The worker's representative did not submit that the worker's range of motion in her back had deteriorated since her 2006 NEL assessment and I was not directed to medical reports indicating this. While the emergency report in February 2012 referred to sciatica, the worker reported experiencing sciatica in 2005, as indicated above.

[67] In these circumstances, I find that the evidence establishes on the balance of probabilities that there was not a measureable change in objective clinical findings since the 2006 NEL assessment. The worker is therefore not entitled to a redetermination of her NEL award for her low back on the basis of increased pain symptoms.

(e) Entitlement to a January 7, 2012 recurrence of the low back injury

[68] Prior to November 1, 2014, Board policy on recurrences (OPM Document No. 15-03-01) stipulates that workers are entitled to benefits for a recurrence of a work-related injury where there is clinical compatibility between the original injury and the current condition, or a combination of clinical compatibility and continuity. This policy goes on to provide further guidelines for evaluating the evidence of clinical compatibility and continuity:

Clinical compatibility

To establish clinical compatibility, a decision-maker compares the worker's current clinical condition to that following the initial accident. The decision-maker considers

- whether the parts of the body affected now are the same as, or related to, those affected initially
- whether the body functions affected now are the same as those affected initially, and
- the degree to which body functions are affected now (as compared to the effect of the initial condition).

Similar clinical conditions indicate that the current problem or problems may be a result of the original injury, whereas dissimilar or unrelated clinical conditions indicate that there is no compatibility, and therefore no recurrence.

Continuity

To establish continuity (i.e., a connection between the original clinical condition and the most recent problem or problems), the decision-maker considers whether the worker has

- complained to supervisors, co-workers, or health care practitioners on an ongoing basis since the original injury
- demonstrated ongoing symptoms since the original injury
- required work restrictions or job modifications
- had ongoing treatment for the original condition, or
- experienced a lifestyle change since the original accident (e.g., has the worker become unable to participate in household duties, or social or recreational activities?).

[69]

The final question in determining entitlement for a recurrence is whether there was a “significant new accident”. If there was a significant new accident, then the condition is not considered a recurrence of a prior workplace injury. The policy sets out the following “equations” as guidelines for adjudicators:

Insignificant new accident + compatibility + continuity = recurrence

An insignificant accident is one involving an ordinary or routine event, such as stooping to fasten a shoe or reaching for something on a shelf. If the accident is insignificant and clinical compatibility and continuity are established, the worker is entitled to benefits for a recurrence of the original injury.

Significant new work-related accident = new claim

A significant accident is one of some consequence or importance, such as a fall from a ladder. If there is a significant new work-related accident, a new claim is established.

A new claim is also established if the worker's latest clinical problem or problems result entirely from a new accident.

[70]

The Policy further states as follows with respect to recurrences on or after January 1, 1998 which occur after the final LOE review date:

In general, the WSIB only pays loss of earnings (LOE) benefits for recurrences that occur within 72 months from the date of injury.

Exceptions

...

- If after the 72-month period expires, the worker suffers a significant deterioration in his or her condition that
 - results in a redetermination of the degree of permanent impairment
 - results in an initial determination of a permanent impairment
 - is likely, in the WSIB's opinion, to result in a redetermination of the degree of permanent impairment, or
- after the 72-month period expires, the worker suffers a significant temporary deterioration in his or her condition that is related to the injury.

[71] As I noted above in connection with the NEL redetermination issue, the worker's symptoms by January 2012 appeared similar to those she had continued to experience since the date of the accident. There was no measurable change in objective clinical findings since the 2006 NEL assessment. In 2006, the worker was awarded a NEL for her low back condition diagnosed as "low back - mechanical back pain." The worker had also previously reported having sciatica in 2005.

[72] On January 8, 2012, Dr. Cybolski treated the worker at a hospital emergency department and diagnosed "chronic mechanical low back pain with sciatica." It is significant that Drs. Kingwell and Tiedje and massage therapist Ms. Simmons reported severe and chronic low back pain during 2011. In my view, this medical reporting supports a finding that while the worker sought treatment for low back symptoms in January 2012, these were more likely than not a continuation of her ongoing compensable low back condition rather than a recurrence. I was not directed to any other medical opinion to the contrary. Accordingly, I find the worker did not sustain a recurrence of her low back condition in January 2012. Additionally, I note that I earlier found that the worker did not sustain a significant deterioration of her low back condition in January 2012. While I recognize a "significant deterioration" is not noted as a specific requirement under OPM Document No. 15-03-01, "Recurrences," this does appear to be clarified as a requirement under the newer Recurrences policy (OPM Document No. 15-02-05).

[73] I find therefore that the worker does not have entitlement for a January 7, 2012 recurrence of her 2005 low back injury.

DISPOSITION

[74] The appeal is allowed in part as follows:

1. The worker has entitlement for the right hip injury, the right hip surgery of November 27, 2009, and the right hip replacement surgery in 2010. The worker is entitled to a NEL assessment for her permanent right hip impairment.
2. Entitlement to full LOE benefits from January 2010 to October 2010 as originally granted and then rescinded by the Board is restored. As the Board did not recover the overpayment, there is no entitlement to further benefits during this time period.
3. The worker is entitled to full LOE benefits following her hip replacement surgery in October 2010 until her return to work at the accident employer in January 2011.
4. The issue of entitlement to LOE benefits after the worker's return to work in January 2011 and at the final review date in June 2011 is referred back to the Board for further adjudication.
5. The worker is not entitled to a redetermination of the 19% NEL award for her low back impairment.
6. The worker does not have entitlement to a recurrence of her 2005 low back injury in January 2012.

[75] The nature and duration of benefits flowing from this decision will be returned to the Board for further adjudication, subject to the usual rights of appeal.

DATED: September 16, 2020

SIGNED: A. Somerville